

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Metformin 500mg/850mg/1000mg tablets for the treatment of Type 2 Diabetes Mellitus Monitoring and Management

Summary of NICE / NICE CKS Guidelines for Type 2 Diabetes Mellitus Monitoring and Management

NICE Guidelines: Type 2 Diabetes Management (NG28)

- **Diagnosis:** Confirmed by fasting plasma glucose ≥ 7.0 mmol/L, 2-hour post-load glucose ≥ 11.1 mmol/L (OGTT), or HbA1c ≥ 48 mmol/mol ($\geq 6.5\%$).
- **First-Line Treatment:** Metformin is the first-line pharmacological agent for type 2 diabetes in non-obese and obese adults. Lifestyle modification is essential.
- **HbA1c Targets:** Target HbA1c 48 mmol/mol (6.5%) for most patients; individualise target based on age, comorbidities, hypoglycaemia risk, and life expectancy.
- **Monitoring:** Check HbA1c every 3-6 months until stable, then annually. Monitor renal function annually (more frequently if eGFR 30-45). Screen for diabetes complications: annual foot screening, annual eye screening, annual urine albumin, annual kidney function.
- **Cardiovascular Risk Reduction:** Address CV risk factors: hypertension, lipid management, smoking cessation, weight loss, cardiovascular medication (ACE inhibitor, statin) if indicated.

Patient Group Direction

for the supply/administration of

Metformin 500mg/850mg/1000mg tablets

for the treatment of

Type 2 Diabetes Mellitus Monitoring and Management

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply

	medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Type 2 diabetes mellitus in patients inadequately controlled by diet and lifestyle modification alone
Inclusion criteria	Aged 18 years and over Type 2 diabetes mellitus diagnosis confirmed Diet and lifestyle modification attempted/ongoing eGFR ≥ 30 mL/min/1.73m² Able to swallow tablets
Exclusion criteria	Severe renal impairment (eGFR < 30 mL/min/1.73m²) Acute conditions affecting renal function: acute dehydration, acute diarrhoea, vomiting, sepsis, severe infections, acute shock Diabetic ketoacidosis (DKA) Acute alcohol intoxication or chronic alcoholism Hypersensitivity to metformin Acute myocardial infarction or unstable angina in previous 3 months Conditions causing tissue hypoxia (acute heart failure, respiratory failure, severe anaemia)
Cautions	Renal impairment: eGFR 30-45: reduce dose; monitor eGFR every 3-6 months. eGFR 45-60: monitor eGFR annually. Hepatic impairment: caution with increased risk of lactic acidosis Heart failure (decompensated or New York Heart Association class III/IV): avoid or use with extreme caution Surgery/invasive procedures/iodinated contrast media: withhold metformin 48 hours before procedure; resume 48 hours after if renal function stable Elderly patients (>65 years): assess renal function carefully; increased risk of lactic acidosis Vitamin B12 deficiency risk: long-term metformin use may reduce B12 absorption; monitor annually

	Congestive heart failure Severe pulmonary disease Acute pancreatitis
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Metformin 500mg, 850mg, and 1000mg tablets
Legal category	POM
Storage	Store below 25°C in a dry place, protected from light
Route/method of administration	Oral, swallowed whole with water
Dose and frequency	Start: 500mg once daily with food Titrate gradually over 2-4 weeks: increase to 500mg twice daily, then 500mg three times daily Usual maintenance: 500mg BD-TDS (1000-1500 mg/day in divided doses) Maximum: 2000 mg/day in divided doses (500mg QID or equivalent) Take with food to minimise GI side effects Reduce dose if eGFR 30-45 (consult BNF for specific recommendations)
Quantity to be administered and/or supplied	28-84 tablets per month depending on titration phase and maintenance dose
Maximum or minimum treatment period	Ongoing. Check HbA1c every 3-6 months until stable target achieved, then annually. Check renal function annually (more frequently if eGFR 30-45). Annual diabetes review: foot screening, eye screening, urine albumin, blood pressure, lipids, weight.
Adverse effects	Common: GI upset (especially on initiation): nausea, diarrhoea, abdominal pain, loss of appetite, metallic taste disturbance. Usually resolve within 1-2 weeks. Uncommon: Vomiting, constipation, dizziness, headache, hepatitis Rare: Lactic acidosis (serious, life-threatening - increased risk with renal/hepatic impairment, acute illness, dehydration, excessive alcohol) Very rare: Vitamin B12 deficiency (with prolonged use)

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with metformin tablets.
Follow-up advice to be given to patient or carer	Take the tablet with food to reduce stomach upset (nausea, diarrhoea common at start) Titrate slowly as directed: start low and increase gradually over 2-4 weeks to reach full dose Do not stop the medication without consulting your doctor Attend all blood tests as arranged: HbA1c, kidney function, vitamin B12 (if on long-term treatment) Report persistent nausea, diarrhoea, or vomiting to your doctor - may need dose adjustment Maintain diet and exercise: medication works best with lifestyle changes Monitor blood glucose at home if advised (target fasting glucose 5-7 mmol/L) Attend annual diabetes review including eye screening, foot examination, urine test, blood pressure check Inform healthcare professionals that you are taking metformin before surgery, dental work, or contrast imaging Avoid alcohol excess as it increases risk of lactic acidosis

Valid from date	Expiry date		
1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026
Name of professional group signatory	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025